

NAME-AMIT GUPTA

DATE-28/03/2026

R-9

Examination : X-RAY PELVIS WITH BOTH HIP JOINT AP

The bones of pelvis and both hip joint shows normal architecture.

The bones forming hips joints show normal alignment.

The hip joint space is normal on either side.

The articular margins are normal.

No soft tissue mass or calcification is seen.

The pelvic bones are normal.

No evidence of loose bodies or joint effusion is noted.

There is no fracture of any bone.

The sacroiliac joints are normal.

IMPRESSION : Normal skiagram of pelvis with hip joints .

DR.CHETAN SHETH
M.D.,D.M.R.D.,D.M.R.E

DR SUNILKUMAR BHUTKA
DMRD DNB

DR GIRISH MISHRA
M.B.B.S.,D.M.R.E


DR AJIT JAIN
M.B.,D.M.R.D

॥ Om Sai Ram ॥

SADGURU DASKISHIN SAIBABA MANDAL (Regd.)

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Our Division of Medical Unit with Dialysis facility

TAI-MAA DIALYSIS & POLYCLINIC CENTRE

Shree Vallabh Darshan, 'B' Wing, 1st Floor, Above Axis Bank, Poddar Marg, Malad (E), Mumbai - 400 097.
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CERTIFICATION (UK) LIMITED
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QUALITY MANAGEMENT SYSTEM

GRAY, SONOGRAPHY, 2-D ECHO FACILITY AVAILABLE FULLY COMPUTERISED PATHOLOGY LABORATORY
CHARITABLE POLYCLINIC & MEDICAL CENTRE ★ COMPLETE BODY HEALTH CHECKUP DONE HERE

Name : AMIT GUPTA Date : 25-Apr-26

Examination : SONOGRAPHY OF ABDOMEN AND PELVIS

LIVER : Liver is normal in size and echopattern. No focal intra-hepatic lesion detected.
Intra-hepatic biliary radicals are not dilated. Portal vein is normal in calibre.

GALL BLADDER : Gall bladder appears echofree with normal wall thickness.
No evidence of calculi. Common bile duct is not dilated.

PANCREAS : Pancreas is normal in size and echopattern.

SPLEEN : Spleen is normal in size & echopattern.

KIDNEYS : Both kidneys are normal in position, size and outline.
Cortical echotexture is normal and cortico-medullary differentiation of right kidney is maintained.
There is no hydronephrosis/hydroureter/calculus on right side.
There is marked left hydronephrosis with cortical thinning. Left ureter is not dilated.
9-20 mm calculi are seen in left kidney.

RETROPERTONEUM : Visualized parts of retroperitoneum do not reveal any lymphadenopathy.

ASCITES : There is no free fluid in abdomen and pelvis.

URINARY BLADDER : Urinary bladder is normal in wall thickness with clear contents.
No calculus or diverticulum.

IMPRESSION :

Marked left hydronephrosis with left renal calculi, presumably due to left PU junction narrowing.

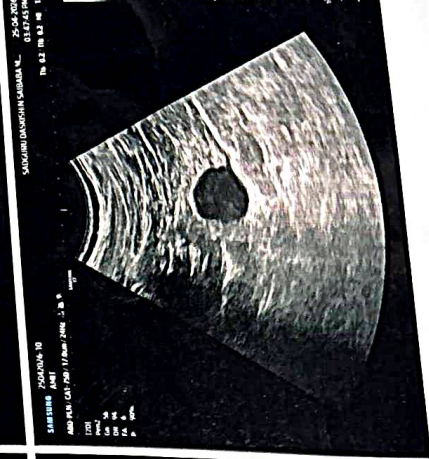
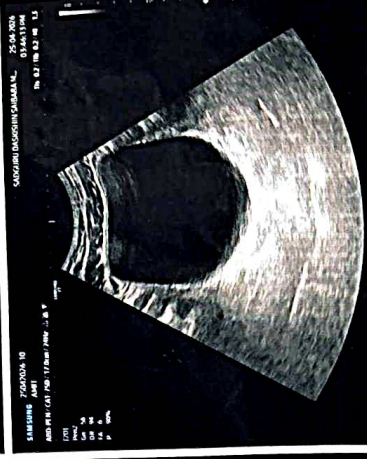
PLEASE CORRELATE WITH CLINICAL AND OTHER INVESTIGATION FINDINGS.

DR. CHETAN SHETH
M.D., D.M.R.D., D.M.R.E

DR. SUNIL KUMAR BHUTKA
DMRD DNB

DR. GIRISH MISHRA
M.B.B.S DMRE

DR. AJIT JAIN
M.B., D.M.R.D



Name : Mr Amit Gupta
Ref By : Dr Badwe Shrikant M

Date : 28-Apr-2026
Age : 27 Years / Sex : M

MDCT SCAN OF KUB WITH UROGRAPHY (Plain & Contrast)

Plain and contrast enhanced MDCT scan of Abdomen & Pelvis with Urography was performed.

Observations:

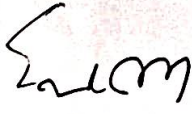
- Enlarged left kidney with moderate to severe hydronephrosis in left kidney with moderate to severe parenchymal thinning in left kidney with dilated extrarenal pelvis showing abrupt focal short segment partial narrowing at the left pelviureteric junction. No enhancing mass lesion is seen at the site of narrowing. No surrounding fat stranding. No extrinsic compression of the pelviureteric junction. Immediate post-contrast study shows normal prompt symmetrical enhancement of the thinned left renal parenchyma. 15 minute delayed scan shows Hypo excretion of contrast in the left pelvicalyceal system partially opacifying the left pelvicalyceal system. 30 minute delayed scan reveals excreted contrast opacifying the entire left pelviureteric junction. The above imaging features are suggestive of partial left pelviureteric junction obstruction with delayed hypofunctioning left kidney. No perinephric fat stranding or collection is seen. AP dimension of the left renal pelvis measuring 4.5 cm. No abnormal wall thickening of pelvicalyceal system. Lower pole parenchymal thickness in the left kidney measuring 6 mm.
- Multiple (3) non-obstructing secondary left renal lower calyceal calculi are seen largest measuring 18 x 10 mm of 1250 HU with the other two calculi measuring 12 mm and 8 mm respectively. Tiny left middle calyx calculus seen measuring 4 mm.
- Extrarenal mildly dilated pelvis is seen in the right kidney without right sided hydronephrosis and no right pelviureteric junction obstruction. AP dimension of right renal pelvis measuring 1.7 cm.
- Right kidney shows normal size, parenchymal thickness, shape and shows normal prompt parenchymal enhancement and excretion.
- There is no evidence of any radio-opaque calculus in right kidney and both ureters. No evidence of any hydronephrosis on right side. No evidence of enhancing mass lesion or cortical scarring.
- Right kidney measures 10.5 x 5.1 cm while left kidney measures 14 x 7.8 cm.
- Ureters appear normal in wall thickness, course and caliber. Both vesico-ureteric junction regions appear normal.
- The urinary bladder is distended . No abnormal wall thickening . No intra-luminal radio-opaque calculus is seen.
- Prostate gland appears normal in size and enhancement. Seminal vesicles are unremarkable.
- Rest of the visualized abdomen reveals no significant major abnormality.

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: Page 2:

Impression:

- Enlarged left kidney with moderate to severe left hydronephrosis with moderate to severe parenchymal thinning in left kidney with dilated extrarenal pelvis showing abrupt focal short segment partial narrowing at the left pelviureteric junction. 15 minute delayed scan shows Hypo excretion of contrast in the left pelvicalyceal system partially opacifying the left pelvicalyceal system. 30 minute delayed scan reveals excreted contrast opacifying the entire left pelvicalyceal system and extending into the left ureter through the partially narrow left pelviureteric junction. The above imaging features are suggestive of partial left pelviureteric junction obstruction with delayed hypofunctioning left kidney.
- Multiple (3) non-obstructing secondary left renal lower calyceal calculi largest measuring 18 mm with the other two calculi measuring 12 mm and 8 mm
- Tiny left middle calyx calculus seen measuring 4 mm.
- Extrarenal mildly dilated pelvis in the right kidney without right sided hydronephrosis and no right pelviureteric junction obstruction.
- Normal functioning right kidney.


Dr. Bharat M Gala
M.D. (Radiology)


Dr. Akshay Nadkarni
M.B.B.S. M.D., DNB, (Radiodiagnosis)

Dr Saurav Date
M.B.B.S., M.D. (Radiodiagnosis)

GUPTA AMIT
Ex: 07 / M
027Y
Study : 009598
Se : 10497 / Im : 1
KUB

04/28/2026 / 22:01:18

GUPTA AMIT
Ex: 07 / M
027Y
Study : 009598
Se : 10548 / Im : 1
KUB

04/28/2026 / 22:01:18

RT SIDE

LT SIDE

Zoom : 1.5

Zoom : 1.5

GUPTA AMIT
Ex: 07 / M
027Y
Study : 009598
Se : 10476 / Im : 1
KUB

04/28/2026 / 22:01:18

GUPTA AMIT
Ex: 07 / M
027Y
Study : 009598
Se : 10455 / Im : 1
KUB

04/28/2026 / 22:01:18

Zoom : 1.5

Zoom : 1.5



Patient Name : MR. AMIT GUPTA

Age / Gender : 27 years / Male

Patient ID : 5850

Source : DIRECT

Referral : Shanti hospital bandra

Collection Date : 12 May, 2026

Reporting Date : 13 May, 2026

Sample ID :



000513326

Test Description	Value(s)	Reference Range	Unit(s)
RENAL(KIDNEY) FUNCTION TEST			
Uric Acid Method : Uricase	6.8	2.30 - 7.60	md/dl
Creatinine Method : Jaffes Kinetic	0.82	0.60-1.20 mg/dL	mg/dL
Calcium Method : Arsenazo III	9.1	8.6-10.2	mg/dL
Phosphorus	2.9	2.50 - 5.0	mg/dl
Sodium Method : Method : ISE(Indirect)	142	135 - 145	mmol/L
Potassium Method : Method : ISE(Indirect)	3.95	3.50 - 5.50	mmol/L
Blood Urea Method : Urease	24	19-45	mg/dL
Chlorides Method : Method : ISE(Indirect)	102	98 - 109	mmol/L
BUN-Blood Urea Nitrogen Method : Serum, Urease	11.21	8 - 20	mg/dL
Urea/Creatinine Ratio	12.39		
BUN/Creatinine Ratio	13.06		

END OF REPORT

Abhinav

Dr.Abhinav Walia
MBBS MD
Consultant Pathologist

Scan to Validate



Patient Name : MR. AMIT GUPTA

Age / Gender : 27 years / Male

Patient ID : 5850

Source : DIRECT

Referral : Shanti hospital bandra

Collection Date : 12 May, 2026

Reporting Date : 13 May, 2026

Sample ID :



000513326

Test Description	Value(s)	Reference Range	Unit(s)
<u>Liver Fuction Test</u>			
Bilirubin Total	0.88	0-1.2	mg/dL
Method : Diazo			
Bilirubin Direct	0.11	0-0.40	mg/dL
Method : Diazo			
Bilirubin Indirect	0.77	0.10 - 1.0	mg/dL
Method : Calculated			
Aspartate Aminotransferase (AST/SGOT)	27	10-40	U/L
Method : IFCC			
Alanine Aminotransferase (ALT/SGPT)	38	0-40	U/L
Method : IFCC			
Alkaline Phosphatase (ALP)	89	36.0 - 113.0	U/L
Method : AMP			
Gamma Glutamyl Transferase (GGT)	18	05 - 40	U/L
Method : IFCC			
Total Protein	8.06	6.4-8.3	g/dL
Method : Biuret			
Albumin	4.89	3.5-5.5	g/dL
Method : Serum, Bromocresol green			
Globulin	3.17	2.3 - 3.5	gm/dL
Method : Calculated			
Albumin/Globulin Ratio	1.54	1.2 - 2.0	Ratio
Method : Calculated			
SGOT/SGPT Ratio	0.71	0.7 - 1.4	Ratio
Method : Calculated			

****END OF REPORT****

Dr. Abhinav Walia

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Consultant Pathologist

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000513326

Test Description	Value(s)	Reference Range	Unit(s)
<u>Urine Routine</u>			
<u>General Examination</u>			
Volume	25		
Appearance	Clear	Clear	
Colour	Pale Yellow	Pale Yellow	
<u>Chemical Examination</u>			
Specific gravity	1.020	1.001 - 1.035	
pH	Acidic 5.5	4.5-7.5	
Glucose	Absent	Absent	
Protein	Negative	Negative	
Bilirubin	Negative	Negative	
Nitrite	Negative	Negative	
Ketones	Negative	Negative	
Leucocyte Esterase	Negative	Negative	
Blood	Absent	Absent	
Urobilinogen	Normal	Normal	
<u>Microscopic Examination</u>			
Pus cells (WBCs)	3-4	0-5	Cells/HPF
Epithelial cells	2-3	0-5	Cells/HPF
RBCs	Nil	Nil	Cells/HPF
Crystals	NIL	Nil	
Cast	Absent	Absent	
Amorphous deposits	Absent	Absent	
Yeast cells	Absent	Absent	
Bacteria	Absent	Absent	
Others (Non Specific)			

END OF REPORT

Dr. Abhinav Walia

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000613326

Test Description	Value(s)	Reference Range	Unit(s)
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CRP Quantitative

C-Reactive Protein; CRP, SERUM **8.43** < 6 mg/L

Method : Serum, Nephelometry

Interpretation :

1. Measurement of CRP is useful for the detection and evaluation of infection, tissue injury, inflammatory disorders and associated diseases .
2. High sensitivity CRP (hsCRP) measurements may be used as an independent risk marker for the identification of individual at risk for future cardiovascular disease.
3. Increase in CRP values are non-Specific and should not be interpreted without a complete history.

****END OF REPORT****

Dr. Abhinav Walia
MBBS MD
Consultant Pathologist

Scan to Validate





SHANTI NURSING HOME & MULTISPECIALTY HOSPITAL



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Tel: 9819554457 / 9987076059 • E-mail: shanthihospital50@gmail.com

Name:	Mr AMIT JITENDRA GUPTA	MR No:	MRSH014822
Age/Gender:	27 Y/M	Visit ID:	IPSH05260002
Address:	002 ASHIRWAD BUILDING NO- 3 B WING DHANJIWADI NEAR HANUMAN TEMPLE RANISATI MARG VTC- MUMBAI MALAD EAST	Admission Date:	01-05-2026 14:00
Location:	MUMBAI, MAHARASHTRA	Discharge Date:	
Doctor:	Dr SHAFEEK AHMED	Ward/Bed:	Twin Sharing/Twin Sharing29
Department:	General Physician	Referred By:	SELF

Discharge Summary

DIAGNOSIS

ACUTE OBSTRUCTIVE UROPATHY WITH CULTURE-PROVEN COMPLICATED URINARY TRACT INFECTION (E. COLI) WITH ACUTE FEBRILE SYSTEMIC INVOLVEMENT, ON BACKGROUND OF LEFT PUJ OBSTRUCTION WITH SEVERE HYDRONEPHROSIS AND MULTIPLE RENAL CALCULI.

SURGERY DONE ON (02/05/2026) AT 10:45 AM - CYSTOSCOPY + LEFT DJ STENTING 1SHORT GA

PRESENTED WITH COMPLAIN

- C/O - FEVER WITH CHILLS SINCE 2 - 3DAYS
- BURNING MICTURITION AND INCREASED FREQUENCY SINCE 3DAYS
- SEVERE LEFT FLANK PAIN SINCE 2 DAYS,
- NAUSEA AND VOMITING SINCE 1 DAY (2 - 3 EPISODES).

HISTORY OF PRESENT COMPLAIN

H/O - PATIENT WAS APPARENTLY ALRIGHT 1 MONTH BACK WHEN HE HAD A HISTORY OF FALL THEN X-RAY SPINE DONE S/O - SUSPICIAN OF RENAL CALCULUS, FURTHER EVALUATION WITH USG KUB S/O - LEFT PELVIC URETERIC CALCULUS WITH HYDRONEPHROSIS, THEN PT CONSULTED TO UROLOGIST ON (27/04/26) AND WAS ADVISED CT KUB SCAN - CONFIRMED LT. PUJ OBSTRUCTION WITH SEVERE HYDRONEPHROSIS, RENAL ENLARGEMENT AND CORTICAL THINNING WITH MULTIPLE RENAL CALCULI PT INITIALLY PRESENTED TO OPD SINCE 2 DAYS BACK AND START ORAL MEDICATION, DESPITE TREATMENT PATIENT DEVELOPED SAME ABOVE COMPLAIN GIVEN WORSENING SYMPTOMS AND SYSTEMIC INVOLVEMENT HENCE ADV - HOSPITALIZATION FOR FURTHER MANAGEMENT,

PAST MEDICAL HISTORY

NO H/O- DM, HTN AND ANY FURTHER COMORBIDITIES

ON EXAMINATION

P: 118/MIN, BP: 100/60MMHG, RR: 22/MIN, SAT: 96%, HGT: 108MG/DL, TEMP: 101°F

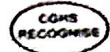
SYSTEMATIC EXAMINATION

CVS: S1, S2(TACHYCARDIA), RS: CLEAR, CNS: CONSCIOUS, P/A: SEVERE TENDERNESS IN LEFT FLANK AND RENAL ANGLE

COURSE DURING HOSPITALISATION



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Tel: 9819554457 / 9987076059 • E-mail: shanthihospital50@gmail.com

IV PCM 1GM (TDS)
IV. EMSET 4MG (TDS)
INJ. BUSCOPAN (1AMP) STAT & SOS
SYP. CITAL 10ML (TDS) WITH ½ GLASS OF WATER
IVF NS + ELDERVIT

INVESTIGATION DONE

ALL RELEVANT INVESTIGATION REPORTS INCLUDING X-RAY CHEST, USG, DT PA SCAN AND LABORATORY PARAMETERS ARE ATTACHED.

DISCHARGE MEDICINE

TAB. CEFTUM 500mg
TAB. FERONEMER-200MG (1--0--1) FOR 5 DAYS
TAB. PAN 40MG (1--0--0) FOR 5 DAYS
TAB. DOLO 650MG (1--0--1) FOR 5 DAYS
CAP. SILOFAST M 8MG/25M (0--0--1) FOR 4 WEEKS
TAB. BEVON (1--0--0) FOR 5 DAYS
SYP. CITAL 10ML WITH 1/2 GLASS OF WATER (1--1--1) FOR 5 DAYS

F/U ON

14-05-2026 (THURSDAY) WITH DR. SHAFEEK AHMED
14-05-2026 (THURSDAY) WITH FOLLOW-UP WITH DR. OJAS POTDAR AFTER 1 WEEK FOR PLANNED LEFT PYELOPLASTY AFTER INFECTION RESOLUTION.

WATCH for COMPLAIN

FEVER
BURNING MICTURITION
NAUSEA, VOMITING

INVESTIGATION ADVISED

CBC, CRP, RFT, LFT, URINE R/M
FURTHER PLAN:
CONTINUE ORAL ANTIBIOTICS
MAINTAIN HYDRATION
REVIEW WITH UROLOGIST AFTER 7-10 DAYS
PLANNED LEFT PYELOPLASTY AFTER INFECTION RESOLUTION

DIET

REGULAR DIET,
AVOID OILY AND SPICY FOOD.
MAINTAIN HYDRATION

Follow Up Details

14-05-2026 Dr. SHAFEEK AHMED
14-05-2026 Dr. Ojas V. Potdar

F/U AFTER 7 DAYS WITH PRIOR APPOINTMENT
FOLLOW-UP WITH DR. OJAS POTDAR AFTER 1 WEEK FOR
PLANNED LEFT PYELOPLASTY AFTER INFECTION
RESOLUTION.

15.05.2025

Mr. Amit Jitendra Gupta.

27 yrs.

C/c

- Swelling Rt forearm. - 1 week.
- Flc UTI on Maxipenm, 300mg BD.
Now No fever.
- Thrombophlebitis Rt forearm.
- Obs. Uropathy. Hydronephrosis (2+)
PUJ stenosis awaiting surgical
management.
- Silofast M 8/25 0-1
4 weeks
- Thrombophlebitis
Flc UTI
Obs. Uropathy.

BP 120/80mmHg

P - 120bpm

RR - 20bpm

SPO2 99%

Soft abdomen
No organ palpable

Right forearm:

Vein palpable

Upper: Supra medial

over

Skin - Normal Color.

Tender to touch

Heart sounds (A)

Inv. 12.05.2026

SUAcd 68

Scr. 0.82mg/l

Adv -

Urine Routine &
Culture Sensitivity.

Renew sdays.

1. Fazopect 300 ER

1-1, 5 days

2. Thrombophobe (Dinh)

Locally Three times
daily

3. Tab. Zerodol SP

1-1 P.C. Sdgs
After food.

4. Zosac 40mg

1-0-0

5. Febuxic 40mg

0-1-0

5 days.

[Signature]